Ysbyty Gwynedd
Anaesthetics Department
Adran Anaestheteg

PERIOPERATIVE CARE

Anaesthesia for Day Case Hip & Knee Arthroplasties

Enhanced Recovery After Surgery
Helping you get better, quicker

YG CG 421 FEB 2021

REVIEW BY FEB 2024

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Reviewer: Day Case Arthroplasty Working Group

It is the responsibility of the prescriber to ensure that there are no contraindications to the following:

Premed:

Longtec® (Oxycodone M/R) 10mg PO at 8 am.

Scopoderm 1.5mg transdermal patch to be applied behind ear if not contra-indicated

Pre-warm patient – Use patients' own clothes, dressing gown, blankets to keep covered until they come to theatre.

Clear fluids until 2 hours pre-operative then sips of water.

Anaesthetic:

GA or Spinal according to patient's preference / medical indication.

For spinal anaesthesia – 15-25mcg of intrathecal Fentanyl can be considered.

Intrathecal Diamorphine should not be used for day case arthroplasty.

Pre-incision:

Antibiotics (IV Cefuroxime 1.5g plus IV Gentamicin <60Kg: 160mg, >60Kg: 240mg at induction only)
- In severe penicillin allergy or current/previous MRSA: IV Teicoplanin (<50Kg: 600mg, >50Kg: 800mg) plus Gentamicin

Dexamethasone 9.9mg IV (dose range 8-10mg)

Tranexamic Acid 1g iv (or 10mg/kg)

Intra-op:

For knee replacement surgery – Adductor canal block should be performed for both spinal and general anaesthesia.

Local anaesthetic infiltration by surgeons – Standard mix = 3 x 50 mls syringes: each containing 20mls of 0.25% bupivacaine with adrenaline plus 30mls saline *(if patient <55kg use same mix but half volume. 55-74kg use same mix but discard 12mls from each syringe)*

Paracetamol 1g (or 15mg/kg if <50kg)
NSAID if no contraindications (IV diclofenac 75mg)
Magnesium Sulphate 5g (20mmol ampoule diluted) infusion during surgery. Titrate rate to BP.
Ondansetron 4mg IV

For patients having a general anaesthetic:
Fentanyl in preference to Morphine (500mcg/10ml Fentanyl is available in theatre recovery).

Inpatient drug chart – Prescribe:

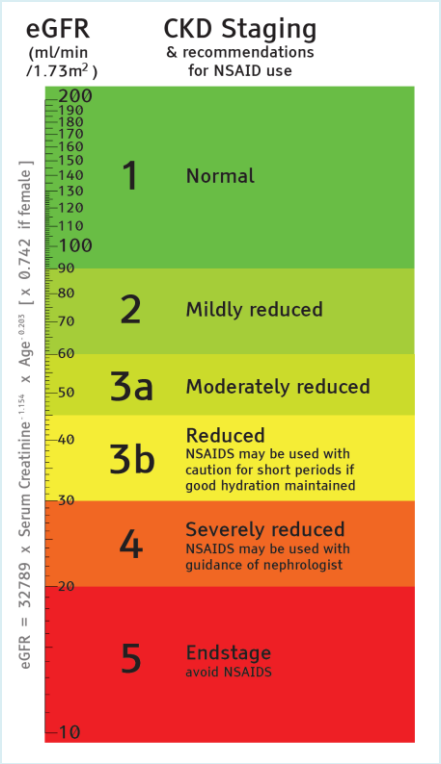
Recovery:

1. Fentanyl 5 x 20mcg boluses for break through pain in recovery *(for GA patients)*

Regular (regardless of the pain scores):

2. Paracetamol 1g QDS IV/PO (15mg/kg if <50kg) *(cross off any doses given in theatre)*

3. Ibuprofen 400mg TDS PO *(cross off any doses given in theatre)*
   - *Celecoxib 200mg BD PO as alternative if H of gastric ulcer or dyspepsia*
   - *Reduce dose and duration in elderly or frail*
   - *Avoid completely only if hypersensitive or eGFR <30ml/min – see chart* →



4. Longtec® (Oxycodone M/R) 10mg PO BD *(next dose will be at 8pm)*
   - *Can be reduced to 5mg BD if patient feels too drowsy with 10mg*

5. Ondansetron 4mg TDS PO

6. Omeprazole 20mg OD PO

7. Lactulose 15ml BD PO

8. Senna 15mg Nocte PO

9. Enoxaparin 40mg s/c NOCTE.
   - Reduce to 20mg if <50kg or CrCl<30ml/min
   - Consider increasing Enoxaparin dose to 60mg if >110kg (unlicensed).

PRN:

10. Shortec® (Oxycodone) 5-10mg PO (Max 2 Hourly) PRN

11. Naloxone 100-400mcg IV PRN

12. Cyclizine 50mg PO/IV TDS PRN

Tudno ward: ***Call Acute Pain team (bleep 115) or Anaesthetist (063) if pain needs reviewing***